



PO No :PO2165710010-237



Name	: Mr.ROHIT AGARWAL	Client Name	: TATA 1MG HYDERABAD
Age/Gender	: 33/Male	Registration Date	: 13/Feb/2025 10:51AM
Patient ID	: HYD486883	Collection Date	: 13/Feb/2025 07:29AM
Barcode ID/Order ID	: D16212892 / 11946834	Report Date	: 13/Feb/2025 12:46PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Whole Blood-EDTA		

HAEMATOLOGY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	15.3	g/dL	13.0-17.0	Cyanide Free SLS
RBC	4.81	10 ⁶ /cu.mm	4.5 - 5.5	Impedance
HCT	44.1	%	40 - 50	Calculated
MCV	91.8	fL	83 - 101	RBC pulse measurement
MCH	31.9	pg	27 - 32	Calculated
MCHC	34.7	g/dL	31.5 - 34.5	Calculated
RDW-CV	15.7	%	11.6-14	Calculated
Total Leucocyte Count	5.95	10 ³ /μL	4 - 10	Impedance
Differential Leucocyte Count				
Neutrophils	58.3	%	40-80	DHSS/Microscopy
Lymphocytes	29.8	%	20-40	DHSS/Microscopy
Monocytes	10	%	2-10	DHSS/Microscopy
Eosinophils	1.8	%	1-6	DHSS/Microscopy
Basophils	0.1	%	0-2	Impedance/ Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count	3.47	10 ³ /μL	2 - 7	Calculated
Absolute Lymphocyte Count	1.77	10 ³ /μL	1-3	Calculated
Absolute Monocyte Count	0.6	10 ³ /μL	0.2 - 1	Calculated
Absolute Eosinophil Count	0.11	10 ³ /μL	0.02 - 0.5	Calculated
Absolute Basophil Count	0.01	10 ³ /μL	0.02-0.1	Calculated
Platelet Count	152	10 ³ /μL	150-410	Impedance /Microscopy
MPV	11.9	fL	6.5 - 12	Calculated
PDW	29.2	fL	9 - 17	Calculated

Comment:

As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.

DHSS : Double Hydrodynamic Sequential System

NABL certificate
and scope

This test has been performed at

TATA 1MG HYDERABAD

Address: SCB Door No. 3-14-011, 1st Floor,
Patny Square, SP Road, Rasoolpura,
Secunderabad, Telangana - 500003

Dr. Vittal Sri Navya
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Patient ID	: HYD486883	Collection Date	: 13/Feb/2025 07:29AM
Barcode ID/Order ID	: D16212892 / 11946834	Report Date	: 13/Feb/2025 04:37PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: EDTA		

HAEMATOLOGY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
Erythrocyte Sedimentation Rate				
Erythrocyte Sedimentation Rate	7	mm/hr	<=10	Modified Westergren

Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis, acute rheumatic fever, Hodgkins disease, temporal arthritis, and systemic lupus erythematosus; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection, malignancy, pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

NABL certificate
and scope

This test has been performed at
TATA 1MG HYDERABAD
Address: SCB Door No. 3-14-011, 1st Floor,
Patny Square, SP Road, Rasoolpura,
Secunderabad, Telangana - 500003

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Patient ID	: HYD486883	Collection Date	: 13/Feb/2025 07:29AM
Barcode ID/Order ID	: D16212892 / 11946834	Report Date	: 13/Feb/2025 01:14PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: WHOLE BLOOD-EDTA		

HAEMATOLOGY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
HbA1c (Glycosylated Hemoglobin)				
Glycosylated Hemoglobin (HbA1c)	5.4	%	4-5.6	HPLC (NGSP certified)
Estimated average glucose (eAG)	108.28	mg/dL	Not established	Calculated

Comment:

Interpretation: HbA1c%

≤5.6	Normal
5.7-6.4	At Risk For Diabetes
≥6.5	Diabetes

Adapted from American Diabetes Association.

Comments:

A 3 to 6 monthly monitoring is recommended in diabetics. People with diabetes should get the test done more often if their blood sugar stays too high or if their healthcare provider makes any change in the treatment plan. HbA1c concentration represent the integrated values for blood glucose over the preceding 8-12 weeks and is not affected by daily glucose fluctuation, exercise & recent food intake.

Please note, Glycemic goal should be individualized based on duration of diabetes, age/life expectancy, comorbid conditions, known CVD or advanced microvascular complications, hypoglycemia unawareness, and individual patient considerations.

Factors that interfere with HbA1c Measurement: Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements.

Factors that affect interpretation of HbA1c Measurement: Any condition that shortens erythrocyte survival or decrease mean erythrocyte age (e. g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c.

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1c result does not correlate with the patient's blood glucose levels.

- HPLC - High performance liquid chromatography

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Patient ID	: HYD486883	Collection Date	: 13/Feb/2025 07:29AM
Barcode ID/Order ID	: D17242150 / 11946834	Report Date	: 13/Feb/2025 02:44PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Fluoride Plasma R		

BIOCHEMISTRY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
RBS (Random Blood Sugar)				
Glucose- Random	80	mg/dL	Normal - 70 - 140,	Hexokinase

Comment:

- Random glucose in plasma measures the glucose levels regardless of the last meal intake.

Ref: American Diabetes Association.



NABL certificate and scope



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Madhuri

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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

BIOCHEMISTRY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
Lipid profile (Non-fasting)				
Cholesterol - Total	196	mg/dL	Low (desirable): < 200 mg/dL Moderate (borderline) 200–239 mg/dL High: >= 240 mg/dL	CHOD-POD
Triglycerides	89	mg/dL	Normal: <150, Borderline: 150 - 199, High:200-499, Very High>=500	GPO
Cholesterol - HDL	47	mg/dL	Undesirable/high risk <40mg/dL Desirable/low risk>=60mg/dl	Accelerator Selective Detergent
Cholesterol - LDL	131	mg/dL	Desirable: <100 Above desirable: 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >=190	Calculated
Cholesterol- VLDL	18	mg/dL	10-30	Calculated
Cholesterol : HDL Cholesterol	4.2	Ratio	Desirable : 3.5-4.5 High Risk : >5	Calculated
LDL : HDL Cholesterol	2.78	Ratio	Desirable : 2.5-3.0 High risk : >3.5	Calculated
Non HDL Cholesterol	149	mg/dl	Desirable:< 130, Above Desirable:130 - 159, Borderline High:160 - 189, High:190 - 219,	Calculated

NABL certificate and scope



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Sample Type	: Serum		

BIOCHEMISTRY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
			Very High: ≥ 220	

Comment:

- Total cholesterol** is the total amount of cholesterol in blood comprising of HDL, LDL-C, and VLDL.
- LDL Cholesterol (LDL-C)** or "bad" cholesterol contributes most significantly to atherosclerosis leading to heart disease or stroke and is the primary target for reducing risk for cardiovascular disease (CVD).
- High-density lipoprotein (HDL-C)** or "good" cholesterol can lower risk of heart disease and stroke.
- Triglyceride (TG)** level also plays a major role in CVD and need treatment. Indians are more prone to atherogenic dyslipidemia, a condition with high TG, low HDL-C and high LDL-C; seen in diabetes, metabolic syndrome and insulin resistance.
- Non-HDL-Cholesterol (Non-HDLC)** measures all plaque forming lipoproteins (e.g. remnants, LDL-C, VLDL, Lp(a), Apo-B).

The Lipid Association of India (LAI) recommends

- Screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors, esp. lipid profile. Risk factors for CVD include: Age (male ≥ 45 years, female ≥ 55 years); Family h/o heart disease at younger age (< 55 yrs in males, < 65 yrs in female), Smoking/tobacco use, High blood pressure, Low HDL (males < 40 mg/dl and females < 50 mg/dl).
- LAI recommends LDL-Cholesterol (LDL-C) as the primary target and Non-HDL-Cholesterol (Non-HDLC) as a co-primary target, for lipid-lowering therapy.
- The Lipid Association of India (LAI-2020) recommends that both fasting and non-fasting lipid profiles are important for managing Indian patients with dyslipidemia. The Non-fasting lipid profile helps to determine post prandial hypertriglyceridemia, blood sampling is simplified for patients and doctors; better for patient follow-up.
- Non-fasting lipid profile is preferred in primary prevention settings, e.g. mass screening of a community; initial lipid profile or CVD risk assessment of a patient if he is non-fasting; emergency assessment of a pancreatitis patient or patients admitted with acute chest pain; in children or elderly; diabetic patients with risk of hypoglycemia.
- A subsequent lipid profile (after 10-12 hrs fasting) may be required if:- Non-fasting triglycerides > 400 mg/dL; known hypertriglyceridemia on treatment or recovering from hypertriglyceridaemic pancreatitis; starting medications that cause severe hypertriglyceridemia; to monitor LDL-C level after starting drug therapy.

Note: Reference Interval as per National Cholesterol Education Program (NCEP) Report

LIVER FUNCTION TEST

Liver Function Test

Bilirubin-Total	0.63	mg/dL	0.3 – 1.2	Vanadate oxidation
Bilirubin-Direct	0.18	mg/dL	0.0-0.3	Vanadate oxidation
Bilirubin-Indirect	0.45	mg/dL	0.1 - 1.0	Calculated

NABL certificate
and scope

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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

BIOCHEMISTRY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
Protein, Total	7.28	g/dL	5.7-8.2	Biuret
Albumin	4.65	g/dL	3.2-4.8	BCG Dye Binding
Globulin	2.6	g/dl	1.8-3.6	Calculated
A/G Ratio	1.77	Ratio		Calculated
Aspartate Transaminase (SGOT)	29	U/L	<34 U/L	Modified IFCC
SGPT (Alanine Transaminase)	25	U/L	10-49	Modified IFCC
SGOT/SGPT	1.16	Ratio		Calculated
Alkaline Phosphatase	64	U/L	46-116	IFCC Standardization
Gamma Glutamyltransferase (GGT)	20	U/L	<73	Modified IFCC

Comment:

- Raised ALT and AST indicate hepatocellular damage (e.g. viral or drugs etc). ALT is more liver-specific while AST is also found in heart, skeletal muscle, and kidney. Mild elevation (less than twice normal) often resolves on its own. Fatty liver disease (especially with metabolic syndrome) is a common cause in asymptomatic cases. Certain drugs (paracetamol, statins), herbal supplements, energy drinks, and antibiotics may also affect liver function.
- SGOT/SGPT Ratio: Typically <1 in healthy individuals (vary between 0.7-1.4; higher in women than men). High SGPT (ratio <1) seen in acute or chronic hepatitis, autoimmune disorders, medications, toxins while ratio >1 indicates alcoholic hepatitis, cirrhosis, metastasis or non-hepatic issues (hemolytic diseases, CVS disorders).
- Elevated Alkaline Phosphatase and GGT: Suggest cholestatic diseases (e.g. bile duct obstruction, primary biliary cirrhosis etc.) and can also be due to bone disease, pregnancy, chronic renal failure, malignancy, and congestive heart failure.
- High Bilirubin: Indicates jaundice due to increased RBC breakdown, liver damage (e.g., infections, toxins), or cholestasis (e.g., gallstones, tumors).
- High Protein Levels: Seen in dehydration (e.g., severe vomiting, diarrhea) or increased production (e.g., inflammation, hematopoietic neoplasms). Low protein and albumin: Result from impaired synthesis (liver disease), decreased intake, tissue damage, malabsorption, or increased renal excretion.

Kidney Function Test.

Blood Urea Nitrogen	8	mg/dL	9.0-23	Urease with GLDH
Urea	17.12	mg/dL	19.26-49.22	Calculated

NABL certificate
and scope

This test has been performed at
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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

BIOCHEMISTRY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
Creatinine	1.33	mg/dL	0.7-1.3	Alkaline picrate-kinetic
Uric Acid	8.5	mg/dL	3.5-7.2	Uricase/Peroxidase
Sodium	138	mEq/L	136-145	Indirect ISE
Potassium	3.89	mEq/L	3.5-5.1	Indirect ISE
Chloride	105.0	mEq/L	98-107	Indirect ISE
BUN/Creatinine Ratio	6.0	Ratio	12:1 - 20:1	Calculated

Comment:

BUN is directly related to protein intake and nitrogen metabolism and inversely related to the rate of excretion of urea. Blood urea nitrogen (BUN) levels reflect the balance between the production and excretion of urea. Increased levels are seen in renal failure (acute or chronic), urinary tract obstruction, dehydration, shock, burns, CHF, GI bleeding, nephrotoxic drugs. Decreased levels are seen in hepatic failure, nephrotic syndrome, cachexia (low-protein and high-carbohydrate diets).

Urea is a non-proteinous nitrogen compound formed in the liver from ammonia as an end product of protein metabolism. Urea diffuses freely into extracellular and intracellular fluid and is ultimately excreted by the kidneys. Increased levels are found in acute renal failure, chronic glomerulonephritis, congestive heart failure, decreased renal perfusion, diabetes, excessive protein ingestion, gastrointestinal (GI) bleeding, hyperalimentation, hypovolemia, ketoacidosis, muscle wasting from starvation, neoplasms, pyelonephritis, shock, urinary tract obstruction, nephrotoxic drugs. Decreased levels are seen in inadequate dietary protein, low-protein/high-carbohydrate diet, malabsorption syndromes, pregnancy, severe liver disease, certain drugs.

Creatinine is catabolic product of creatinine phosphate, which is excreted by filtration through the glomerulus and by tubular secretion. Creatinine clearance is an acceptable clinical measure of glomerular filtration rate (GFR). Increased levels are seen in acute/chronic renal failure, urinary tract obstruction, hypothyroidism, nephrotoxic drugs, shock, dehydration, congestive heart failure, diabetes. Decreased levels are found in muscular dystrophy.

BUN/Creatinine ratio (normally 12:1-20:1) is decreased in acute tubular necrosis, advanced liver disease, low protein intake, and following hemodialysis. BUN/Creatinine ratio is increased in dehydration, GI bleeding, and increased catabolism.

Uric acid levels show diurnal variation. The level is usually higher in the morning and lower in the evening. Increased levels are seen in starvation, strenuous exercise, malnutrition, or lead poisoning, gout, renal disorders, increased breakdown of body cells in some cancers (including leukemia, lymphoma, and multiple myeloma) or cancer treatments, hemolytic anemia, sickle cell anemia, or heart failure, pre-eclampsia, liver disease (cirrhosis), obesity, psoriasis, hypothyroidism, low blood levels of parathyroid hormone (PTH), certain drugs, foods that are very high in purines - such as organ meats, red meats, some seafood and beer. Decreased levels are seen in liver disease, Wilson's disease, Syndrome of inappropriate antidiuretic hormone (SIADH), certain drugs.

Calcium

Calcium	9.8	mg/dL	8.7-10.4	Arsenazo III
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Comment:

Increased in: Hyperparathyroidism primary and secondary, Acute and chronic renal failure, Following renal transplantation, Osteomalacia with malabsorption, Acute osteoporosis, Malignant tumours (specially of breast, lung and kidney), Drugs: Vit. D and A intoxication, Diuretics, estrogen, androgen, tamoxifen, lithium

NABL certificate and scope



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Sample Type	: Serum		

BIOCHEMISTRY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
Decreased in: Hypoparathyroidism, Surgical and Idiopathic, Pseudohypoparathyroidism, Chronic renal disease with uremia and phosphate retention, Malabsorption of Calcium and Vit.D, obstructive jaundice, Bone Disease (Osteomalacia and rickets), Drugs: Cancer chemotherapy drugs, calcitonin, loop-actives diuretics, Hypomagnesemia, Hypoalbuminemia				

Iron Studies, Basic

Iron Serum	87	µg/dL	65-175	Ferrozine
Unsaturated Iron Binding Capacity	235	µg/dL	120-470	Ferene
Total Iron Binding Capacity (TIBC)	322	µg/dL	240-450	Calculated
Transferrin Saturation	27.02	%	16 - 50	Calculated

Comment:

Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron is seen in iron deficiency and anaemia of chronic disorders. Increased iron concentration are seen in hemolytic anaemias, hemochromatosis and acute liver disease. Serum Iron alone is unreliable due to considerable physiologic diurnal variation in the results with highest values in the morning and lowest values in the evening as well as variation in response to iron therapy .

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. Increased levels of TIBC suggest that total iron body stores are low, increased concentration may be the sign of Iron deficiency anaemia, polycythemia vera ,and may occur during the third trimester of pregnancy. Decreased levels may be seen in hemolytic anaemia, hemochromatosis, chronic liver disease, hypoproteinemia ,malnutrition.

Unsaturated Iron Binding Capacity (UIBC) is increased in low iron state and decreased in high iron concentration such as hemochromatosis. In case of anaemia of chronic disease the patient may be anaemic but has adequate iron reserve and a low UIBC.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

IMMUNOLOGY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
Immunoglobulin E (IgE) Total	228	IU/mL	0 - 158	CLIA

Comment:

- Immunoglobulin E (IgE) is the most important trigger molecule for allergic information.
- As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease.
- The level of IgE is low during the first year of life, gradually increases with age and reaches adult level after 10 years.

Uses

- For Allergy testing.
- Evaluation of children and adults suspected of having allergic respiratory disease
- To confirm clinical expression of sensitivity to foods in patients with Anaphylactic sensitivity or with Asthma, Angioedema or Cutaneous disease.
- To confirm the presence of IgE antibodies to certain occupational allergens

Increased Levels:

Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Bronchopulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases, Hodgkin's disease, etc.

Decreased Levels:

Hereditary deficiencies, Acquired immunodeficiency, Ataxia Telangiectasia, Non IgE Myeloma

Note:

Normal levels of IgE does not eliminate the possibility of allergic diseases
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Thyroid profile Total

T3, Total	0.88	ng/mL	0.60 - 1.81	CLIA
T4, Total	8.5	µg/dl	4.5 - 10.9	CLIA
Thyroid Stimulating Hormone - Ultra Sensitive	1.938	uIU/ml	0.55 - 4.78	CLIA

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IMMUNOLOGY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
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Comment:

- Below mentioned are the guidelines for pregnancy related reference ranges for TSH, total T3 & Total T4.

Pregnancy			
	TSH (μ IU/mL) (as per American Thyroid Association)	Total T3 (ng/mL)	Total T4(μ g/dL)
1st trimester	0.1-2.5	0.81-1.90	7.33-14.8
2nd trimester	0.2-3.0	1.00-2.60	7.93-16.1
3rd trimester	0.3-3.0	1.00-2.60	6.95-15.7

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm .
- The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
- TSH is secreted in a dual fashion: Intermittent pulses constitute 60-70% of total amount, background continuous secretion is 30-40%.These pulses occur regularly every 1-3 hrs.
- Total T3 & T4 concentrations are altered by physiological or pathological changes in thyroxine binding globulin (TBG) capacity .
- The determination of free T3 & free T4 has the advantage of being independent of changes in the concentrations and binding properties of the binding proteins.
- Changes in thyroid status are typically associated with concordant changes in T3, T4 and TSH levels.
- Unexpectedly abnormal or discordant thyroid test values may be seen with some rare, but clinically significant conditions such as central hypothyroidism, TSH-secreting pituitary tumors, thyroid hormone resistance, or the presence of heterophilic antibodies (HAMA) or thyroid hormone autoantibodies.
- For diagnostic purposes, results should be used in conjunction with other data.

TSH	T3	T4	Interpretation
High	Normal	Normal	Subclinical Hypothyroidism

NABL certificate
and scope

This test has been performed at
TATA 1MG HYDERABAD
Address: SCB Door No. 3-14-011, 1st Floor,
Patny Square, SP Road, Rasoolpura,
Secunderabad, Telangana - 500003

Dr. K Madhuri
MBBS, MD (Pathology)
Consultant Pathologist
Reg. No: 80149

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PO No :PO2165710010-237



Name	: Mr.ROHIT AGARWAL	Client Name	: TATA 1MG HYDERABAD
Age/Gender	: 33/Male	Registration Date	: 13/Feb/2025 10:51AM
Patient ID	: HYD486883	Collection Date	: 13/Feb/2025 07:29AM
Barcode ID/Order ID	: D17242152 / 11946834	Report Date	: 13/Feb/2025 12:32PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

IMMUNOLOGY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
Low	Normal	Normal	Subclinical Hyperthyroidism	
High	High	High	Secondary Hyperthyroidism	
Low	High/Normal	High/Normal	Hyperthyroidism	
Low	Low	Low	Non thyroidal illness / Secondary Hypothyroidism	

Vitamin D (25-OH)

Vitamin D (25-OH)	31.7	ng/ml	Deficiency:< 20, Insufficiency:20-29, Sufficiency:30 - 100, Toxicity possible:> 100	CLIA
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Comment:

- Vitamin D is a fat-soluble steroid prohormone involved in the intestinal absorption of calcium and the regulation of calcium homeostasis.
- Two forms of vitamin D are biologically relevant - vitamin D3 (Cholecalciferol) and vitamin D2 (Ergocalciferol).
- Both vitamins D3 and D2 can be absorbed from food but only an estimated 10-20perc. of vitamin D is supplied through nutritional intake.
- Vitamin D is converted to the active hormone 1,25-(OH)₂-vitamin D (Calcitriol) through two hydroxylation reactions. The first hydroxylation converts vitamin D into 25-OH vitamin D and occurs in the liver. The second hydroxylation converts 25-OH vitamin D into the biologically active 1,25-(OH)₂-vitamin D and occurs in the kidneys as well as in many other cells of the body.
- Most cells express the vitamin D receptor and about 3perc. of the human genome is directly or indirectly regulated by the vitamin D endocrine system.
- The major storage form of vitamin D is 25-OH vitamin D and is present in the blood at up to 1,000 fold higher concentration compared to the active 1,25-(OH)₂-vitamin D. 25-OH vitamin D has a half-life of 2-3 weeks vs. 4 hours for 1,25-(OH)₂-vitamin D. Therefore, 25-OH vitamin D is the analyte of choice for determination of the vitamin D status.
- Risk factors for vitamin D deficiency include low sun exposure, inadequate intake, decreased absorption, abnormal metabolism, vitamin D resistance and liver or kidney diseases.
- Vitamin D deficiency is a cause of secondary hyperparathyroidism and diseases resulting in impaired bone metabolism (like rickets, osteomalacia).

NABL certificate and scope



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Secunderabad, Telangana - 500003

Madhuri

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IMMUNOLOGY

TAX SAVER HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Interval	Method
- Recently, many chronic diseases such as cancer, high blood pressure, osteoporosis and several autoimmune diseases have been linked to vitamin D deficiency. - The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D				

Utility Quantitative determination of 25-hydroxyvitamin D (25-OH vitamin D).



NABL certificate and scope



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Madhuri

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Age/Gender	: 33/Male	Registration Date	: 13/Feb/2025 10:51AM
Patient ID	: HYD486883	Collection Date	: 13/Feb/2025 07:29AM
Barcode ID/Order ID	: D16212894 / 11946834	Report Date	: 13/Feb/2025 12:37PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Urine		

CLINICAL PATHOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
Urine Routine & Microscopy				
Urine Routine & Microscopy				
Colour	Pale Yellow		Pale Yellow	Manual
Appearance	Clear		Clear	Manual
Specific gravity	1.005		1.003 - 1.035	pKa change
pH	6.0		4.6 - 8.0	Double Indicator
Glucose	Negative		Negative	GOD-POD
Protein	Negative		Negative	Protein Error Principle
Ketones	Negative		Negative	Nitroprusside
Blood	Negative		Negative	Peroxidase
Bilirubin	Negative		Negative	Diazonium
Urobilinogen	Normal		Normal	Ehrlich
Leucocyte Esterase	Negative		Negative	Pyrrole
Nitrite	Negative		Negative	P-arsanilic acid
Pus cells	1-2	/hpf	0-5	Microscopy
Red Blood Cells	Nil	/hpf	0-2	Microscopy
Epithelial cells	1-2	/hpf	Few	Microscopy
Casts	Nil	/lpf	Nil	Microscopy
Crystals	Nil		Nil	Microscopy
Yeast	Nil		Nil	Microscopy
Bacteria	Nil		Nil	Microscopy

Comment:

•Note: Pre-test condition to be observed while submitting the sample-first void, mid stream urine, collected in a clean, dry, sterile container is recommended for routine urine analysis, avoid contamination with any discharge from vaginal, urethra, perineum, Avoid prolonged transit time & undue exposure to sunlight.

•During interpretation, points to be considered are Negative nitrite test does not exclude the urinary tract infections. Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs. • Urine microscopy is done in centrifuged urine specimens

*** End Of Report ***

NABL certificate
and scope

This test has been performed at
TATA 1MG HYDERABAD
Address: SCB Door No. 3-14-011, 1st Floor,
Patny Square, SP Road, Rasoolpura,
Secunderabad, Telangana - 500003

Dr. Vittal Sri Navya
MBBS, MD (Pathology)
Consultant Pathologist
Reg. No: 85499

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PO No :PO2165710010-237



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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Urine		

CLINICAL PATHOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
Conditions of Laboratory Testing & Reporting: Test results released pertain to the sample, as received. Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the interpreting clinician. Result delays may happen because of unforeseen or uncontrollable circumstances. Test report may vary depending on the assay method used. Test results may show inter-laboratory variations. Test results are not valid for medico-legal purposes. Please mail your queries related to test results to Customer Care mail ID care@1mg.com Disclaimer: Results relate only to the sample received. Test results marked "BOLD" indicate abnormal results i.e. higher or lower than normal. All lab test results are subject to clinical interpretation by a qualified medical professional. This report cannot be used for any medico-legal purposes. Partial reproduction of the test results is not permitted. Also, TATA 1mg Labs is not responsible for any misinterpretation or misuse of the information. The test reports alone may not be conclusive of the disease/condition, hence clinical correlation is necessary. Reports should be vetted by a qualified doctor only.				



NABL certificate and scope



This test has been performed at
TATA 1MG HYDERABAD
Address: SCB Door No. 3-14-011, 1st Floor,
Patny Square, SP Road, Rasoolpura,
Secunderabad, Telangana - 500003

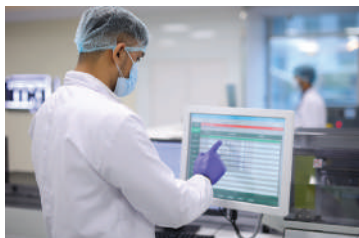
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Thorough third-party assessment by authorized experts



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Ensuring smooth sample collection experience & pre-analytical precision

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SCHEDULE NOW >



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Get Second Opinion



Order Medicines



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1000 mg



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Saturated Fats*



Every batch
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^ Stark KD, et al, progress in Lipid Research 2016 | * Compared with competing products

+ Reference from NCBI published article on comparative bioavailability | # Optimal requirement of Omega-3 as mentioned by National Institutes of Health, Dietary Supplement Label Database, 2015.